جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : هدي حمدي عبد الستار السيد
المحافظة : الفيوم **الحي / المدينة :** اطسا **العنوان :** ببب
رقم البطاقة : 28506302300948 **رقم التليفون :** 01091763847 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 30-06-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: هدي حمدي عبد الستار السيد
National ID: 28506302300948

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

مصر هي الصحة

MDT FORM 18/12/2022 G

Name	هدى حمدى عبدالستار السيد	MRN	٤٩٠٣
		ID	٢٨٥٠٦٢٠٢٤٠٠٩٢٨
Age	38y	Phone	01091763847
Menopausal status	Premenopausal	Marital status	Married
NRS SCORE		Family history	-
BMI		REASON OF MDT	
CO morbidity	-		

Clinical Data

RT

BX: Breast Core biopsy & IDC G2

ER:	PR:	HER 2:	Ki67:
8/8	6/8	+1	10%

Sonomammography: US UOQ 3x2cm + Ax LNs skin thickening

Mets Workup: CT scans Bilateral pulm nodules HFL + (L) iliac bone sclerotic lesion

R L

TNM

Recommendations: Bone scan, Triphasic CT Abdomen

Meanwhile start hormonal tt Tamoxifen with no clinical Crises

MDT OUTCOME & PLAN

Zoladex - femara. received 1st dose

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. ياسر أنور د. نرمين مصطفى					

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: هدى حمدي عبد الستار	
MR. : 5614	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 42-year-old female diagnosed with de novo metastatic breast cancer involving the liver and an iliac sclerotic bony lesion. She was started on a combination of Femara (letrozole) and Zoladex (goserelin) with a calcium supplement (CT CAP) beginning in July 2025. The regimen was administered subcutaneously: Femara 1 mg daily, Zoladex 10.8 mg SC, and calcium 1 g daily.

On 29 April 2026, the pharmacy confirmed the pending supply of Femara, calcium, and Zoladex for the patient.

On 14 July 2026, the oncology team noted that the patient was continuing Femara and Zoladex therapy. The patient received the medications today for a planned three-month course, and an annual breast sonogram was scheduled.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **ايه مدحت بدوي محمد**

يعتمد ,

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m5600

اسم المريض	: هدى حمدى عبدالستار السيد	النوع	: Female
السـن	: 37 Y	العنوان	: الشيخ زايد
تاريخ اخذ العينة	:	تاريخ استلام العينة	: 23/10/2022 09:24
طالب التحليل	: مستشفى الشيخ زايد التخصصى	تاريخ التقرير	: 30/10/2022 09:31
عناية	: مستشفى الشيخ زايد التخصصى		

Primary Sample :

Paraffin Block

Test :

ER &PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / XT .
Sample recived was treated by :- CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: strong nuclear staining of 90 % of tumour cells
PR: moderate nuclear staining of 60 % of tumour cells
HER2/neu protein over expression : discontinuous membrane staining of tumor cells.
Ki67 :positive nuclear staining of about 10 % of tumour cells

Referal Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 8 / 8)
PR: POSITIVE (SCORE 6 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+)
#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 10 % .
(CORE BIOPSY)

Pathologist: د/أيناس رمزى

Lab Director: د/تريزا ماهر

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email:pathologychl@yahoo.com
facebook: قسم الباثولوجى - الادارة المركزية للمعامل

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يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name: Hoda Hamdy Abd Al Sattar Al Sayed	Date : 18/02/2025
Exam : C.T CHEST	Age : 39 Year(s)
Medical ID: 4903/2022	Pat. ID: 30800

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- **Compared to last study dated 24/08/2024.**

I-Chest:

- **Stationary course regarding:**
  - Right breast skin thickening and retro areolar irregular speculated mass lesion with skin retraction.
  - Enlarged right axillary level I and II lymph nodes the largest measures 1 cm.
  - Prominent left axillary lymph nodes, largest still measures 2x1 cm.
  - Still seen few bilateral pulmonary nodules, the largest is noted at inferior lingular segment measuring 5 mm (likely non-specific nodules).
- No pathological hilar or mediastinal lymph nodes.
- Stationary calcified subcarinal lymph node likely old granuloma.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.

II-Abdomen and pelvis:

- The liver is average in size and the previously mentioned right hepatic lobe ill-defined hypodense area can't be appreciated in today's studyb.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الأشعة

Patient Name: Hoda Hamdy Abd Al Sattar Al Sayed	Date : 18/02/2025
Exam : C.T CHEST	Age : 39 Year(s)
Medical ID: 4903/2022	Pat. ID: 30800

- Normal appearance of the pancreas, and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs. IUD seen in uterus.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- Still seen left iliac wing sclerotic lesion.
- Still seen subtle lucent foci affecting multi-vertebral levels.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing:

- **Overall stationary course.**

Dr.Shaimaa Megahed,MSc

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

????, ??? **Patient ID: 10110224006125 DOB: 03-Jun-1985 Date: 12-Aug-2024**
Patient Lastname: هدى
Patient Firstname: هدى
HIS Patient ID: 10110224006125
RIS Patient ID: 357395
Date of Birth: 03/06/1985

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Dual Phase Bone Scan Report (12/08/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits seen all over the surveyed skeleton.
" Degenerative changes over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion: -
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Mahmoud Akram
Dr. Omnia Talaat, MD

========== REPORT TEXT ENDE ==========

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name:	**Huda Hamdi Abdel Sattar El-Sayed**
Date:	**2026-08-24**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Patient with history of right breast cancer. Compared to the previous study report dated 12/4/2026:

Respiratory motion artifact degrading the study quality.

I-CHEST:



- Still noted right breast scarring vs soft tissue with skin thickening and nipple retraction for correlation with sonomammography.
- Still noted right axillary LN measuring 11x10mm.
- Mild mosaic parenchymal attenuation with no evidence of sizable pulmonary masses, nodules or cavitations.



- Subcarinal calcified LN measuring 13x9mm.
- No pathological enlarged lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels

II-ABDOMEN AND PELVIS

- Average size of the liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No definite cysts, masses or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name:	**Huda Hamdi Abdel Sattar El-Sayed**
Date:	**2026-08-24**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- Normal appearance of the pelvic organs with no collections. IUD seen in place.
- No suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- **Bone window still shows left iliac bone sclerotic lesion.**



OPINION :

Follow up study of a case of right breast cancer, current study reveals:

- **Right breast scarring vs soft tissue with skin thickening and nipple retraction associated with right axillary LN for correlation with sonomammography.**
- **No suspicious pulmonary nodules.**
- **left iliac bone sclerotic lesion for correlation with bone scan.**

Shohenda. M. Taha, MD

[[DOCTYPE:BONE_SCAN]]
Bone Scan

Name : Huda Hamdi Abd Al-Sattar El-Sayed **Date : 03-09-2026 10:09**
Branch : Dokki **Medical No. : 453487**
Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 41-year-old female with right breast carcinoma. Presents with occasional bone aches. Referred for follow-up evaluation compared to prior study dated August 12, 2024.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Homogeneous symmetric physiological tracer distribution throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Mild symmetrical periarticular uptake is noted in shoulders and knees, consistent with degenerative changes—stable compared to the prior study.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases. Stable, compared to prior study (August 12, 2024).

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER